

Patient Fall Prevention and Post-Fall Response

Policy Number	POL-PS-002	Owner / Department	Nursing / Patient Safety
Effective Date	2023-09-01	Version	3.4
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Approved By	Patient Safety & Quality Committee	Applies To	All inpatient and observation units, the ED, and procedural areas

1. Purpose

To reduce patient falls through risk assessment and prevention, and to standardize the clinical response, monitoring, and reporting after a fall.

2. Scope

Applies to all patients in inpatient, observation, emergency, and procedural settings. Fall-risk assessment is completed on admission, every shift, after any fall, and on change in condition.

3. Definitions

- **Fall** — an unplanned descent to the floor or other lower surface, with or without injury.
- **Witnessed / unwitnessed fall** — whether a staff member directly observed the descent.
- **Head strike** — any impact to the head, or an unwitnessed fall in which head impact cannot be excluded.

4. Post-Fall Immediate Response

IMPORTANT — Do not move the patient until injury has been assessed. A fall with head strike, or any fall in a patient on anticoagulant or antiplatelet therapy, is SEV-1 and requires physician notification within 1 hour and immediate consideration of head CT.

1. Stay with the patient; call for help. Assess airway, breathing, circulation, and level of consciousness.
2. Assess for injury head-to-toe before moving — pain, deformity, bleeding, and signs of fracture or spinal injury.
3. Obtain vital signs and a neurological assessment; determine whether head strike occurred or cannot be excluded.
4. Notify the attending provider — within 1 hour for any head strike, anticoagulated patient, or suspected injury; promptly for all other falls.
5. Initiate neuro checks when indicated (see Section 6) and notify the patient's family per the patient's wishes.
6. Conduct a post-fall huddle and file an event report per POL-RM-013.

5. Neurological Monitoring After Head Strike

For any head strike, unwitnessed fall where head impact cannot be excluded, or anticoagulated patient, initiate neurological checks and vital signs on the following minimum schedule (increase frequency for any deterioration):

- Every 15 minutes × 4 (first hour),
- then every 1 hour × 4,
- then every 4 hours to complete 24 hours of monitoring.

Anticoagulated and antiplatelet patients carry elevated risk of delayed intracranial hemorrhage; maintain a low threshold for head CT even without external signs of trauma, and re-notify the provider for any new neurological change.

6. Severity Classification

Finding	Severity
Head strike, anticoagulated patient, or intracranial hemorrhage / fracture / major injury	SEV-1
Fall with injury requiring treatment (e.g., laceration requiring sutures, minor fracture)	SEV-2
Fall with no injury	SEV-3

7. Documentation Requirements

- Whether the fall was witnessed; activity and location at time of fall.
- Whether head strike occurred or cannot be excluded; anticoagulant/antiplatelet status.
- Assessment findings, vital signs, and neuro-check results with times.
- Provider and family notification (who, when); interventions and imaging ordered.

8. References

- AHRQ, Preventing Falls in Hospitals: A Toolkit for Improving Quality of Care.
- The Joint Commission, National Patient Safety Goals — falls.
- Related: POL-RM-013 (severity & reporting).